

Bipolar Disorder - Self-Care

CCG: C-0061 (CCG)

[Link to Codes](#)

MCG Health
Chronic Care
28th Edition

- Assessment
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Assessment

Initial Consideration

Q1. Patient: What would you like to talk about today? (Check all that apply.) **QM**

- Emergency or urgent problems (eg, social determinants of health, intimate partner violence, suicidal ideation)^[A] Social Determinants of Health Screening Assessment [CCG](#) Intimate Partner Violence [CCG](#) Suicide or Personal Harm Risk [CCG](#)
- Condition information
- Provider coordination (eg, next appointment, treatment clarification) Appointment Scheduling [CCG](#)
- Medications
- Complications
- Monitoring and management
- Other health problems (eg, comorbid conditions, new symptoms)
- Other, specify: _____
- Nothing
- Unknown

Q2. Patient: Do you have any changes since the last time we talked? (Check all that apply.) **QM**

- Adhering to treatment or medication problems Readiness to Change [CCG](#) Medication Adherence [CCG](#)
- Administering medication Home Infusion Management [CCG](#) Injectable Medications [CCG](#)
- Affording medicine or supplies Financial Status and Benefits [CCG](#)
- Aspiration Aspiration Risk Management [CCG](#)
- Behavioral health provider appointment, specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Blood pressure care Blood Pressure Monitoring [CCG](#)
- Blood sugar care Blood Glucose Monitoring [CCG](#) HbA1c Monitoring [CCG](#)
- Blood thinner care Anticoagulation Management [CCG](#)
- Bowel care Bowel Management [CCG](#)
- Bowel changes Bowel Symptoms [CCG](#)
- Caregiver Caregiver Resources Evaluation [CCG](#)
- Chemotherapy Chemotherapy [CCG](#)
- Cognitive impairment Cognitive Impairment [CCG](#) Behavioral Health and Cognition Evaluation with Caregiver [CCG](#)

- Communication or speech problems [Speech and Communication Problems](#)  CCG
- Community resource [Community Resources](#)  CCG
- Dehydration care [Dehydration Monitoring and Management](#)  CCG
- Depression symptoms [Depression Screening, Adult](#)  CCG
- Dialysis care [Dialysis Management](#)  CCG
- Doctor or specialist outpatient appointment, specify: _____ [Follow-Up - Provider Appointment](#)  CCG
- Energy level changes [Energy Level Changes](#)  CCG
- Enteral nutrition care [Enteral Nutrition Management](#)  CCG
- Exercising and staying active [Exercise](#)  CCG
- Fatigue or weakness [Fatigue or Weakness](#)  CCG
- Fluid restriction [Fluid Restriction](#)  CCG
- Functional status changes [Activities of Daily Living](#)  CCG [Instrumental Activities of Daily Living](#)  CCG
- Home infusion [Home Infusion Management](#)  CCG
- Hospital or emergency room visit, specify: _____ [Hospitalization Risk Assessment](#) 
 [CCG Post-Hospitalization Follow-Up](#)  CCG
- Medication changes [Medication Changes](#)  CCG
- Nebulizer care [Nebulizer Management](#)  CCG
- Neurologic function changes [Neurologic Function Changes](#)  CCG
- Ostomy care [Ostomy Management](#)  CCG
- Oxygen care [Oxygen Management](#)  CCG
- Pain, chronic [Chronic Pain](#)  CCG
- Pain, new or general [Pain Assessment](#)  CCG
- Parenteral nutrition care [Parenteral Nutrition Management](#)  CCG
- Peak flow meter care [Peak Flow Meter Use](#)  CCG
- Peripheral vascular (eg, lower leg redness or swelling) [Peripheral Vascular Problems](#)  CCG
- Physical impairments [Physical Impairments](#)  CCG
- Planning for long-term care needs [Advance Care Planning](#)  CCG
- Procedure, specify: _____ [Follow-Up - Provider Appointment](#)  CCG
- Pulmonary treatments [Pulmonary Treatments](#)  CCG
- Rehabilitation therapy, specify: _____ [Follow-Up - Provider Appointment](#)  CCG
- Respiratory changes [Respiratory Symptoms](#)  CCG
- Safety [Safety](#)  CCG
- Scheduling care [Appointment Scheduling](#)  CCG
- Sleeping problems [Sleep](#)  CCG
- Social support [Social Support](#)  CCG
- Stasis ulcer care [Stasis Ulcer Management](#)  CCG
- Testing (eg, laboratory, imaging), specify: _____ [Follow-Up - Provider Appointment](#) 
 CCG
- Tracheostomy care [Tracheostomy Management](#)  CCG
- Transportation [Transportation](#)  CCG
- Understanding healthcare needs [Health Literacy](#)  CCG
- Urinary catheter care [Urinary Catheter Management](#)  CCG
- Urinary changes [Urinary Symptoms](#)  CCG
- Venous thromboembolism symptoms [Venous Thromboembolism](#)  CCG
- Ventilator care [Ventilator Management](#)  CCG
- Weight gain [Nutritional Status, Adult](#)  CCG [Nutritional Status, Pediatric](#)  CCG [Nutritional Status, Infant](#)  CCG
- Weight loss [Nutritional Status, Adult](#)  CCG [Nutritional Status, Pediatric](#)  CCG [Nutritional Status, Infant](#)  CCG

- Wound care Wound Management  CCG
- Other, specify: _____
- No
- Unknown Hospitalization Risk Assessment  CCG

Condition Knowledge

Q1. Patient: What causes bipolar disorder? (Check items understood.)

- Actual cause unknown
- Brain chemical imbalance
- Family history (genetics)
- Other, specify: _____
- Unknown [P1]
- Nothing [P1]
- Not all appropriate information known [P1]

Q2. Case Manager: Rate the patient's understanding of the causes of bipolar disorder. **QM**

- 90% or more
- 75% to 89% [P1]
- 50% to 74% [P1]
- Less than 50% [P1]
- Unknown [P1]

Q3. Patient: What are the symptoms of bipolar disorder? (Check items understood.)

- Excessive risky behavior (eg, foolish business investments, sexual promiscuity, shopping sprees)
- Feeling very irritable
- Feeling you can do things you cannot really do
- Hallucinations, like seeing things that are not there or hearing voices
- Jumping from one thought to another quickly
- Manipulative behavior
- Talking very fast
- Trouble at school or work
- Trouble concentrating
- Trouble making decisions
- Trouble remembering things
- Trouble sleeping
- Very strong mood changes from being too happy or excited to sad or feeling like everything is hopeless
- Other, specify: _____
- Unknown [P1]
- Nothing [P1]
- Not all appropriate information known [P1]

Q4. Case Manager: Rate the patient's understanding of the symptoms of bipolar disorder. **QM**

- 90% or more
- 75% to 89% [P1]
- 50% to 74% [P1]
- Less than 50% [P1]
- Unknown [P1]

Q5. Patient: How can you care for bipolar disorder to keep it from getting worse and causing problems? (Check items understood.)

QM

- Adhere to self-care plan
- Ask for help when feelings are overwhelming
- Avoid things that make your symptoms worse
- Exercise
- Follow a daily care plan
- Follow an eating (diet) plan
- Get all follow-up laboratory tests when due
- Keep a diary about feelings, activities, eating times, and medicines taken
- Keep all doctor appointments
- Keep a vaccination schedule
- Know when to contact the doctor
- Manage situation or event triggers

- Manage stress
- Reduce caffeine intake
- Set realistic expectations for feeling better, managing symptoms, and recovery
- Stop using alcohol, tobacco, or drugs
- Take medicines as ordered
- Other, specify: _____
- Unknown [P1]
- Nothing [P1]
- Not all appropriate information known [P1]

Q6. Case Manager: Rate the patient's understanding of how to care for bipolar disorder. **QM**

- 90% or more
- 75% to 89% [P1] [P2]
- 50% to 74% [P1] [P2]
- Less than 50% [P1] [P2]
- Unknown [P1] [P2]

Q7. Patient: What activities would you be able to do if you didn't have problems with your bipolar disorder? (Check all that apply.) **QM**

- Live independently [P2]
- Go out with friends [P2]
- Make and keep friends [P2]
- Have a relationship [P2]
- Repair broken relationships [P2]
- Travel [P2]
- Work [P2]
- Other, specify: _____ [P2]
- Unknown [P1]

Provider Information

Q1. Patient: What doctor manages your bipolar disorder? (Check all that apply.) **QM**

- Psychiatrist or psychologist, name and phone number: _____ [Q2]
- Behavioral health therapist, name and phone number: _____ [Q3]
- Primary doctor, name and phone number: _____
- Other specialist, name and phone number: _____ [Q5]
- Don't have doctor [P1]
- Unknown [P1]

Q2. Patient: When was your last visit with the psychiatrist or psychologist?

- Less than 1 month ago
- 1 to 3 months ago
- 4 to 6 months ago [P2]
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]
- Not applicable

Q3. Patient: When was your last visit with a behavioral health therapist?

- Less than 1 month ago
- 1 to 3 months ago [P2]
- 4 to 6 months ago [P2]
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]
- Not applicable

Q4. Patient: When was your last visit to your primary doctor? **QM**

- Within the past 3 months
- 4 to 6 months ago
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]

Q5. Patient: When was your last visit to the specialist doctor?

- Within the past 3 months
- 4 to 6 months ago [P2]
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]
- Not applicable

Q6. Patient: What other specialists have you seen within the last 12 months? (Check all that apply.) **QM**

- Behavioral health provider
- Cardiologist
- Dentist
- Dietitian
- Endocrinologist
- Gastroenterologist
- General surgeon
- Genetic counselor
- Geriatrician
- Gynecologist
- Hematologist
- Hepatologist
- HIV/AIDS
- Infectious disease
- Nephrologist
- Neurologist
- Neurosurgeon
- Obstetrician
- Oncologist
- Ophthalmologist
- Orthopedic surgeon
- Orthopedist
- Otolaryngologist
- Pain management
- Physiatrist
- Plastic surgeon
- Podiatrist
- Pulmonologist
- Rheumatologist
- Transplant specialist
- Urologist
- Other, specify: _____
- None [P1]
- Unknown [P1]

Q7. Patient: How many bipolar disorder-related emergency department visits or hospital admissions have you had in the past 12 months? **QM**

- 2 or more [P2] Hospitalization Risk Assessment [CCG](#)
- 1 [P2]
- None
- Other, specify: _____
- Unknown [P2]

Q8. Patient: Do you have a written self-care plan from your doctor on how to care for bipolar disorder?[B] **QM**

- Yes [Q9]
- No [P1] [P2]
- Unknown [P1] [P2]

Q9. Patient: What are the components of your bipolar disorder self-care plan? (Check all that apply.) **QM**

- Action plan for when symptoms start
- Attending regular therapy or counseling appointments
- Crisis management plan
- Exercise plan

- Knowing what medicines you take and why and how to take them
- Managing side effects of medicines
- Nutrition plan
- Recognizing the symptoms of gradual worsening or problems
- Regular follow-up appointments for wellness checks
- When to call the doctor and when to get emergency help
- Other, specify: _____ [P1]
- Unknown [P2]
- Not applicable
- Not all appropriate information known [P2]

Q10. Case Manager: Is the patient able to understand and adhere to the self-care plan? **QM**

- Yes, independently
- Yes, with occasional reminders or caregiver support
- Yes, with frequent reminders or caregiver support
- No [P3]
- Unknown [P3]

Medication Information

Q1. Patient: Has any medicine for bipolar disorder been prescribed by your doctor? If so, is it taken as ordered? **QM**

- Yes, and taken as ordered [Q2]
- Yes, but not taken as ordered [Q2] [P4] Medication Adherence [CCG](#)
- No medicine prescribed [P4]
- Unknown [P4]

Q2. Patient: What medicines do you take for bipolar disorder? (Check all that apply.) **QM**

- Anticonvulsants (eg, clonazepam, lamotrigine, valproic acid) [Q3] [P5] [P6]
- Antidepressants (eg, citalopram, sertraline) [Q3] [P5] [P7]
- Atypical antipsychotics (eg, aripiprazole, clozapine, risperidone, ziprasidone) [Q3] [P5] [P8]
- Mood stabilizers (eg, carbamazepine, divalproex sodium, lithium) [Q3] [P5] [P9]
- Other, specify: _____ [Q3] [P4] [P5]
- Not applicable [P4]
- Unknown [P5]

Q3. Patient: Are you having any side effects from the bipolar disorder medicine? (Check all that apply.)

- Confusion [P5]
- Dark stools or hard stools (constipation) [P5]
- Dizziness [P5]
- Dry mouth [P5]
- Nausea or vomiting [P5]
- Not able to urinate [P5]
- Sexual dysfunction (abnormal ejaculation, impotence, or loss of orgasm) [P5]
- Sleepiness [P5]
- Tremors or twitching [P5]
- Weight loss or gain [P5]
- Other, specify: _____ [P5]
- None
- Unknown [P5]
- Not applicable

Q4. Case Manager: Rate the patient's understanding of the purpose and side effects of the medications for bipolar disorder. **QM**

- 90% or more
- 75% to 89% [P1] [P5]
- 50% to 74% [P1] [P5]
- Less than 50% [P1] [P5]
- Unknown [P1] [P5]

Q5. Patient: Do you take any medicine that may make your bipolar disorder worse? (Check all that apply.)

- Amiodarone [P3] [P4]
- Antibiotics [P3] [P4]
- Antidepressants [P3] [P4]

- Barbiturates [P3] [P4]
- Cimetidine [P3] [P4]
- Cisplatin [P3] [P4]
- Diltiazem [P3] [P4]
- Diuretics [P3] [P4]
- Metformin [P3] [P4]
- Methyldopa [P3] [P4]
- Metoprolol or propranolol [P3] [P4]
- NSAIDs [P3] [P4]
- Oral contraceptives [P3] [P4]
- Prescription pain medicines [P3] [P4]
- Theophylline [P3] [P4]
- Other, specify: _____ [P3] [P4]
- None
- Unknown [P3] [P4]

Complications

Q1. Patient: What problems with bipolar disorder have you had in the past month? (Check all that apply.)

- Feeling bad or angry at yourself [P2] [P3]
- Feeling confused or not thinking clearly [P2] [P3]
- Feeling that you can do things that you really cannot do [P2] [P3]
- Feeling very irritable [P2] [P3]
- Grades dropping in school or having problems taking tests [P2] [P3]
- Not doing well with assignments at work [P2] [P3]
- Sleeping problems [P2] [P3]
- Starting to drink alcohol or using drugs [P2] [P3]
- Talking very fast and jumping quickly from one thought to another [P2] [P3]
- Trouble concentrating or remembering things [P2] [P3]
- Very strong changes in mood like being too happy or excited [P2] [P3]
- Very strong changes in mood like being too sad or feeling like everything is hopeless [P2] [P3]
- Other, specify: _____ [P2] [P3]
- None
- Unknown [P2]

Q2. Patient: Has your mood changed over the past 2 weeks?(1)(2)  PHQ-2 Calculator **QM**

- Yes [Q3]
- No
- Unknown

Q3. Patient: What changes in your mood have you had? (Check all that apply.) **QM**

- Change in usual activity (eg, working excessively, no longer talking to friends, or going on shopping sprees) Depression Screening, Adult  CCG
- Down, hopeless, or depressed Depression Screening, Adult  CCG
- Forgetting a lot or getting angry or frustrated Depression Screening, Adult  CCG
- Little interest or pleasure in doing things Depression Screening, Adult  CCG
- Quick mood changes Depression Screening, Adult  CCG
- Other, specify: _____ Depression Screening, Adult  CCG
- Not applicable

Q4. Patient: Are there any current sleeping problems? (Check all that apply.) **QM**

- Falling asleep [P10]
- Staying asleep [P10]
- Waking up too early [P10]
- Not feeling rested after sleeping [P10]
- Other, specify: _____ [P10]
- None
- Unknown [P10]

Q5. Patient: What do you do to help sleep? (Check all that apply.)

- Decrease caffeine
- Don't eat big meals before bedtime
- Get daily exercise
- Go to bed and get up at routine times of the day
- Keep journal of thoughts
- Sleep in bed, not on chair or sofa
- Sleep in dark, quiet room
- Sleep with head elevated, not lying flat
- Stopped drinking
- Stopped smoking
- Take a warm bath or shower before bedtime
- Take medicines, specify: _____ [Q6]
- Took distractions out of bedroom (eg, TV)
- Use equipment (eg, oxygen or CPAP)
- Other, specify: _____
- Nothing
- Unknown [P10]
- Not all appropriate sleep promoting activities being used [P10]

Q6. Patient: Do the medicines work to help sleep?

- Yes
- No [P10]
- Unknown
- Not applicable

Q7. Patient: Are there any safety risks that may lead to problems being safe at home? (Check all that apply.)

- Accessibility (eg, outside or inside stairs) [Q8] [P11]
- Dizziness [Q8] [P11]
- Falls [Q8] [P11]
- Inside home safety hazards (eg, halls and rooms are cluttered, inadequate lighting, bathroom unsafe, stairs without rails, pets) [Q8] [P11]
- Memory or cognition [Q8] [P11]
- Outside home safety hazards (eg, doors that do not lock, stairs without rails) [Q8] [P11]
- Polypharmacy (4 or more medicines) [Q8] [P11]
- Problems with mobility or transfers [Q8] [P11] Activities of Daily Living  CCG
- Substances used that increase risk (eg, benzodiazepines, sedative-hypnotics, antidepressants, antipsychotics, alcohol, opiates)
- Vision problems [Q8] [P11]
- Other safety concerns: _____ [Q8] [P11]
- None
- Unknown [Q8]

Q8. Patient: Is there a plan in place to manage home safety risks? If yes, what is the plan?

- Yes, specify: _____ [P11]
- No [P12]
- Unknown [P11]
- Not applicable

Q9. Patient: Are you concerned about your safety?

- Yes, specify: _____ [P11] [P12] Intimate Partner Violence  CCG
- No
- Unknown [P11]

Q10. Patient: Has a nurse or physical therapist ever come to your home to do a home safety assessment?

- Yes, specify date: _____
- No [P11]
- Unknown [P11]

Q11. Case Manager: Rate the patient's understanding of the potential complications of bipolar disorder. **QM**

- 90% or more
- 75% to 89% [P1] [P2]
- 50% to 74% [P1] [P2]

- Less than 50% [P1] [P2]
- Unknown [P1] [P2]

Monitoring and Management

Q1. Patient: How much do you weigh and how tall are you? **QM**

- Weight: _____
- Height: _____
- Unknown [P13]

Q2. Case Manager: What is the patient's body mass index (BMI)?  BMI Calculator

- Less than 18.5 [P13] [P14] Nutritional Management to Gain Weight  CCG Body Weight Planner  CCG
- 18.5 to 24.9
- 25 to 29.9 [P13] [P14] Nutritional Management to Lose Weight  CCG Body Weight Planner  CCG
- 30 or greater [P13] [P14] Nutritional Management to Lose Weight  CCG Body Weight Planner  CCG
- Unknown [P13]

Q3. Patient: Do you know what your body weight should be?

- Yes, it should be: _____
- No [P13]
- Unknown [P13]

Q4. Patient: What do you eat and drink in a typical day? (Check all reported.)

- Alcohol, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Dairy foods, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Fats, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Fruits, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Fruit juices, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Grains, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Meats (beef, chicken, fish), specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Milk, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Sodas, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Supplements, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Sweets, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Vegetables, specify types and number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Water, specify number of daily servings: _____ [Q5] [Q6] [Q7] [Q8] [Q9]
- Other, specify: _____ [Q5] [Q6] [Q7] [Q8] [Q9] [P13]
- No food by mouth [Q9] [P13] [P14]
- Unknown [Q5] [Q6] [Q7] [Q8] [Q9] [P13] [P14]

Q5. Patient: How often do you eat each day? (Check all reported.) **QM**

- 3 meals or more [P13] [P14]
- 2 meals
- 1 meal [P13] [P14]
- 0 meals [P13] [P14]
- 2 snacks or more [P13] [P14]
- 1 snack
- 0 snacks
- Other, specify: _____ [P13] [P14]
- Not applicable
- Unknown [P13] [P14]

Q6. Patient: How much fluid (such as water, juice, coffee, tea, milk) do you have per day? **QM**

- Less than 3 cups (720 mL) [P13] [P14]
- 3 to 5 cups (720 mL to 1.2 L) [P13] [P14]
- More than 5 cups (1.2 L)
- Unknown [P13] [P14]

Q7. Patient: Are you trying to gain or lose weight?

- Trying to gain weight [P13] Nutritional Management to Gain Weight  CCG Body Weight Planner  CCG

- Trying to lose weight [P13] Nutritional Management to Lose Weight [CCG](#) Body Weight Planner [CCG](#)
- Neither
- Unknown [P13]

Q8. Patient: Does your doctor want you to follow a special diet program? (Check all that apply.) **QM**

- Yes, DASH (low sodium) [P13] Diet Knowledge Insufficient - DASH Diet [CCG](#)
- Yes, heart healthy [P13] Diet Knowledge Insufficient - Heart Healthy [CCG](#)
- Yes, kidney diet [P13] Diet Knowledge Insufficient - Renal Dysfunction [CCG](#)
- Yes, low carb or low glycemic index [P13] Diet Knowledge Insufficient - Low Glycemic Index [CCG](#)
- Yes, pregnancy diet [P13] Diet Knowledge Insufficient - Pregnancy [CCG](#)
- Yes, other: _____ [P13]
- No [P13]
- Unknown [P13]

Q9. Patient: Do you get nutritional treatments or have an altered diet? If yes, why? (Check all that apply.) **QM**

- Behavioral health provider seen, specify: _____ [P14]
- Dental soft diet, specify: _____ [P14]
- Diet supplements, specify: _____ [P14]
- Dietitian seen or following, specify: _____ [P14]
- Enteral nutrition (tube feeding), specify: _____ [P14] Enteral Nutrition Management [CCG](#)
- IV fluids (hydration), specify: _____ [P14]
- Liquid diet, specify: _____ [P14]
- Mechanical soft diet, specify: _____ [P14]
- Parenteral nutrition, specify: _____ [P14] Parenteral Nutrition Management [CCG](#)
- Procedure(s) done by a doctor in office, specify: _____ [P14]
- Pureed diet, specify: _____ [P14]
- Speech-language pathology (SLP), specify: _____ [P14]
- Supervised during nutritional intake, specify: _____ [P14]
- Surgery, specify: _____ [P14]
- Thickened liquid diet, specify: _____ [P14]
- Other, specify: _____ [P14]
- No
- Unknown [P13]

Q10. Patient: Do you know how to read food labels?

- Yes
- No [P15]
- Unknown [P15]

Q11. Patient: Do you get regular exercise? **QM**

- Yes
- No [P2]
- Unknown [P2]

Q12. Patient: Is there someone you talk to about your problems?

- Yes, name and relationship: _____
- No [B1] [P16]
- Unknown [B1] [P16]

Q13. Patient: Do you have someone to count on when you need help?

- Yes, name and relationship: _____
- No [B1] [P16]
- Unknown [B1] [P16]

Q14. Patient: Is there someone who can help you feel better when you are stressed, sad, or upset?

- Yes, name and relationship: _____
- No [B1] [P16]
- Unknown [B1] [P16]

Q15. Patient: Is there someone who accepts you for who you are, the good and bad?

- Yes, name and relationship: _____
- No [B1] [P16]
- Unknown [B1] [P16]

Q16. Patient: Is there someone you can count on to care about you no matter what is happening?

- Yes, name and relationship: _____
- No [B1] [P16]
- Unknown [B1] [P16]

Q17. Patient: Does your social support help you as much as you need? **QM**

- Yes
- No [B1] [P16]
- Unknown [B1] [P16]

Q18. Patient: Have additional treatments, procedures, or therapy been considered? (Check all that apply.) **QM**

- Electroconvulsive therapy (ECT), date scheduled: _____ [P2]
- Other, specify: _____ [P2]
- None
- Unknown [P2]

Q19. Patient: Do you participate in a program for bipolar disorder? (Check all that apply.)

- Behavioral health counseling
- Community education program
- Support group
- Exercise program
- Not now, but in the past [Q20] [P2] [P3]
- Other, specify: _____
- No [P2] [P3]
- Unknown [P2] [P3]

Q20. Patient: When was the last time you participated in a therapy program?

- 3 months ago or less
- 4 to 11 months ago
- 12 months ago or more [P2]
- Other, specify: _____ [P2]
- Unknown [P2]
- Not applicable

Q21. Patient: Are you having any problems with your bipolar disorder self-care needs? (Check all that apply.) **QM**

- Affording medicine or supplies [P2] [P3] Financial Status and Benefits [CCG](#)
- Behavior indicating cognitive impairment [P2] [P3] Cognitive Impairment [CCG](#) Behavioral Health and Cognition Evaluation with Caregiver [CCG](#)
- Blood sugar care [P2] [P3] Blood Glucose Monitoring [CCG](#)
- Caregiver help needed [P2] [P3] Caregiver Resources Evaluation [CCG](#)
- Community resource information needed [P2] [P3] Community Resources [CCG](#)
- Daily care of child [P2] [P3] Caregiver Resources Evaluation [CCG](#)
- Exercising and staying active [P2] [P3] Exercise [CCG](#)
- Taking medicine as prescribed [P2] [P3] Medication Adherence [CCG](#)
- Transportation for care [P2] [P3] Transportation [CCG](#)
- Trouble gaining weight [P2] [P3] Nutritional Management to Gain Weight [CCG](#)
- Trouble losing weight [P2] [P3] Nutritional Management to Lose Weight [CCG](#)
- Trouble making lifestyle changes [P2] [P3] Readiness to Change [CCG](#)
- Trouble understanding how to do self-care [P2] [P3] Health Literacy [CCG](#)
- Other, specify: _____ [P2] [P3]
- No
- Unknown [P2] [P3]

Q22. Case Manager: Rate the patient's understanding of the monitoring and management of bipolar disorder. **QM**

- 90% or more
- 75% to 89% [P1] [P2]
- 50% to 74% [P1] [P2]
- Less than 50% [P1] [P2]
- Unknown [P1] [P2]

Other Health Considerations

Q1. Patient: How often did you have a drink with alcohol in the past year?[C] **QM**

- Never
- Monthly or less often [Q2]
- 2 to 4 times per month [Q2]
- 2 to 3 times per week [Q2]
- 4 or more times per week [Q2]
- Unknown

Q2. Patient: How many drinks did you have on a typical day in the past year? **QM**

- None
- 1 to 2
- 3 to 4 [P17] Alcohol Misuse [CCG](#)
- 5 to 6 [P17] Alcohol Misuse [CCG](#)
- 7 to 9 [P17] Alcohol Misuse [CCG](#)
- 10 or more [P17] Alcohol Misuse [CCG](#)
- Unknown

Q3. Patient: How often did you have 6 or more drinks on one occasion in the past year? **QM**

- Never
- Less than monthly
- Monthly [P17] Alcohol Misuse [CCG](#)
- Weekly [P17] Alcohol Misuse [CCG](#)
- Daily or almost daily [P17] Alcohol Misuse [CCG](#)

Q4. Patient: Do you have any risk factors for alcohol misuse? (Check all that apply.) **QM**

- Comorbid behavioral health diagnosis (eg, depression, posttraumatic stress disorder) Alcohol Misuse [CCG](#)
- Family history Alcohol Misuse [CCG](#)
- Life-changing event (eg, death of a loved one, chronic condition diagnosis) Alcohol Misuse [CCG](#)
- Traumatic experience in childhood Alcohol Misuse [CCG](#)
- Other, specify: _____ Alcohol Misuse [CCG](#)
- None
- Unknown

Q5. Patient: In the past 4 weeks, which of these substances have you used? Please specify how many times the substance was used. (Check all that apply.) **QM**

- ADHD drugs not prescribed for a medical condition, frequency of use: _____ [P18]
- ADHD drugs used more than prescribed, frequency of use: _____ [P18]
- Amphetamines, frequency of use: _____ [P18]
- Anxiety drugs not prescribed for a medical condition, frequency of use: _____ [P18]
- Anxiety drugs used more than prescribed, frequency of use: _____ [P18]
- Cannabinoids, synthetic, frequency of use: _____ [P18]
- Cathinones, synthetic, frequency of use: _____ [P18]
- Cocaine, frequency of use: _____ [P18]
- Desomorphine, frequency of use: _____ [P18]
- Flunitrazepam (Rohypnol), frequency of use: _____ [P18]
- Hallucinogens, such as LSD (lysergic acid diethylamide), mescaline (peyote), or psilocybin (mushrooms), frequency of use: _____ [P18]
- Heroin, frequency of use: _____ [P18]
- Inhalants (huffing), frequency of use: _____ [P18]
- Insomnia drugs not prescribed for a medical condition, frequency of use: _____ [P18]

- Insomnia drugs used more than prescribed, frequency of use: _____ [P18]
- Kratom, frequency of use: _____ [P18]
- Marijuana (cannabis) not prescribed for a medical condition, frequency of use: _____ [P18]
- Marijuana (cannabis) used more than prescribed, frequency of use: _____ [P18]
- MDMA (3,4-methylenedioxymethamphetamine (ecstasy)), frequency of use: _____ [P18]
- Methamphetamine, frequency of use: _____ [P18]
- Narcotic drugs not prescribed for a medical condition, frequency of use: _____ [P18]
Narcotic/Opioid Misuse Assessment [CCG](#)
- Narcotic drugs prescribed for medical condition, frequency of use: _____ [P18]
Narcotic/Opioid Misuse Assessment [CCG](#)
- PCP (phencyclidine), frequency of use: _____ [P18]
- Salvia, frequency of use: _____ [P18]
- Steroids, frequency of use: _____ [P18]
- Other, specify: _____ [P18]
- None
- Unknown [P18]

Q6. Patient: How long have you been using illegal substances or misusing legal drugs (drugs that are not prescribed for you or not taken as ordered)? **QM**

- Never have used illegal substances or misused legal drugs
- Less than 1 year [Q7] [P19]
- 1 to 3 years [Q7] [P19]
- Longer than 3 years [Q7] [P19]
- Unknown [Q7] [P18]

Q7. Patient: When was the last time you were high?

- Specify: _____ [P18]
- Unknown [P18]

Q8. Patient: Do you currently smoke or use tobacco products? (Check all that apply.) **QM**

- Yes, I smoke. [Q9] [Q11] [Q12] [Q13]
- Yes, I use other tobacco product; specify: _____ [Q10] [Q11] [Q12] [Q13]
- No, I quit. [Q11] [Q14]
- No, I have never smoked or used other tobacco products.
- Unknown

Q9. Patient: How much do you smoke?

- 2 packs a day or more [P20]
- More than 1 but less than 2 packs a day [P20]
- 1 pack a day [P20]
- Less than 1 pack a day [P20]
- Unknown
- Not applicable

Q10. Patient: How often do you use other tobacco products?

- 3 or more times a day [P20]
- 1 or 2 times a day [P20]
- A few times a week [P20]
- A few times a month [P20]
- A few times a year [P20]
- Unknown [Q13]
- Not applicable

Q11. Patient: How old were you when you started smoking or using tobacco products?

- Specify age: _____
- Unknown

Q12. Patient: What triggers you to want to smoke or use tobacco? (Check all that apply.)

- After meals [P20]

- Alcohol use [P20]
- Being around people who are smoking [P20]
- Driving or talking on the phone [P20]
- Stress [P20]
- Other, specify: _____ [P20]
- Nothing [P20]
- Unknown [P20]
- Not applicable

Q13. Patient: Are you ready to stop smoking or using tobacco products? **QM**

- Yes [P20]
- No [P20] Readiness to Change  CCG
- Unknown [P20] Readiness to Change  CCG

Q14. Patient: When did you quit smoking or using tobacco products?

- More than 1 year ago [P21]
- Within the last year [P21]
- Unknown

Q15. Patient: What vaccinations have you had? (Check all that apply.) **QM**

- Coronavirus disease 2019 (COVID-19) [Q16]
- *Haemophilus influenzae* type b (Hib) [Q16] [Q17]
- Hepatitis A [Q16]
- Hepatitis B [Q16]
- Human papillomavirus (HPV) [Q16]
- Influenza [Q16] [Q17]
- Measles, mumps, rubella (MMR) [Q16]
- Meningococcal 4-valent conjugate (MenACWY) or polysaccharide (MPSV4) [Q16]
- Meningococcal B (MenB) [Q16]
- Pneumococcal 13-valent conjugate (PCV13) [Q16] [Q18]
- Pneumococcal 15-valent conjugate (PCV15) [Q16] [Q18]
- Pneumococcal 20-valent conjugate (PCV20) [Q16] [Q18]
- Pneumococcal 23-valent polysaccharide (PPSV23) [Q16] [Q18]
- Polio (inactivated) [Q16]
- Respiratory syncytial virus (RSV) [Q16]
- Rotavirus [Q16]
- Tetanus booster [Q16]
- Tetanus, diphtheria, pertussis (Td/Tdap) [Q16]
- Varicella [Q16]
- Zoster [Q16]
- Other, specify: _____ [Q16]
- None, because: _____ [P22]
- Unknown [P22]

Q16. Patient: Are you up to date with the vaccinations you need (for age, booster status, or medical condition)? **QM**

- Yes
- No, because: _____ [Q19] [P22]
- Unknown [Q19] [P22]

Q17. Patient: When was your last flu shot? **QM**

- In the past year
- More than a year ago [Q19] [P22]
- Never [Q19] [P22]
- Unknown [Q19] [P22]

Q18. Patient: When was your last pneumonia shot?

- In the last year
- 2 to 4 years ago
- 4 to 5 years ago [Q19] [P22]
- More than 5 years ago [Q19] [P22]
- Never [Q19] [P22]

- Unknown [Q19]

Q19. Patient: Do you have an appointment scheduled to get caught up on your vaccinations?

- Yes
- No [P22]
- Unknown [P22]

Q20. Patient: Do you have other conditions that might make your bipolar disorder worse? (Check all that apply.) **QM**

- Alcohol or substance misuse [P2] [P3] Substance-Related Disorders - Self-Care [CCG](#)
- Anxiety [P2] [P3] Anxiety - Self-Care [CCG](#)
- Attention-deficit hyperactivity disorder [P2] [P3] Attention-Deficit Hyperactivity Disorder, Adult - Self-Care [CCG](#)
- Coronary artery disease [P2] [P3] Coronary Artery Disease - Self-Care [CCG](#)
- Diabetes [P2] [P3] Diabetes, Adult - Self-Care [CCG](#)
- High cholesterol [P2] High Cholesterol - Self-Care [CCG](#)
- HIV/AIDS [P2] [P3] HIV/AIDS - Self-Care [CCG](#)
- Obesity or overweight [P2] [P3] Obesity, Adult - Self-Care [CCG](#)
- Pancreatitis [P2] [P3]
- Physical inactivity [P2] [P3] Exercise [CCG](#)
- Pregnancy [P2] [P3] High-Risk Pregnancy - Self-Care [CCG](#)
- Other, specify: _____ [P2] [P3]
- No
- Unknown [P1]

Patient Understanding Evaluation

Q1. Case Manager: Does the patient understand all the causes of bipolar disorder?

- Yes
- No [P1]
- Unknown [P1]

Q2. Case Manager: Does the patient understand all the symptoms of bipolar disorder?

- Yes
- No [P1]
- Unknown [P1]

Q3. Case Manager: Does the patient understand how to care for bipolar disorder? **QM**

- Yes
- No [P1] [P2]
- Unknown [P1] [P2]

Q4. Case Manager: Does the patient know when and where to get help for problems?

- Yes
- No [P1] [P3]
- Unknown [P1] [P3]

Q5. Case Manager: Does the patient have the ability to understand and follow the self-care plan? **QM**

- Yes
- No [P3]
- Unknown [P3]

Q6. Case Manager: Does the patient understand the purpose and side effects of their bipolar disorder medication?

- Yes
- No [P5]
- Unknown [P5]

Q7. Case Manager: Is the patient having trouble coping with their bipolar disorder? **QM**

- Yes [P3]
- No
- Unknown [P3]

Q8. Case Manager: Does the patient agree to education? **QM**

- Yes
- No, explain why: _____ [P3] Readiness to Change  CCG
- Unknown [P3]

Quality Measures

- Disease Management Quality Measures:
 - Information provided from this question may be requested by various accrediting organizations.
- HEDIS Measures include(3):
 - Tobacco use is assessed and addressed.
 - Readmission risk is assessed and addressed.
 - Alcohol and substance misuse is assessed and addressed.

Plan of Care

Barriers

B1. Inadequate social support

Problems

P1. Bipolar disorder information needed

- **Goal:** Bipolar disorder diagnosis better understood(4)(5)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **EDUCATE: Definition:** Bipolar disorders are a group of mood disorders characterized by unusual shifts in mood, energy, activity levels, and ability to carry out day-to-day tasks. It is also called manic-depressive disorder.(6)(7)
 - Changes in mood may be gradual or rapid, and vary by individual. It is a severe lifelong illness that often leads to functional impairment and disability.(6)
 - It can cause impairments of personal, educational, social, and occupational functioning.
 - **EDUCATE: Causes(6):**
 - Exact cause is unknown.
 - Family history (genetics)
 - Psychosocial and environmental influences
 - Traumatic event history
 - **EDUCATE: Symptoms(7)(8):**
 - Risky behaviors (eg, gambling, shopping sprees, promiscuity)
 - Behavior problems (eg, aggression, manipulation)
 - Mood instability (eg, extreme or rapid changes)
 - Hallucinations or delusions
 - Cognitive changes (eg, confusion, memory loss)
 - Concentration difficulty
 - Thought process problems (eg, planning, reasoning, decision making)
 - Depression
 - Impulse control problems
 - Interest in activities or hobbies lost
 - Sleep problems
 - **EDUCATE: Diagnosis:**
 - Based on medical history, risk factors, physical examination, and diagnostic tests
 - Behavioral health examination and history
 - **EDUCATE: Treatment(9)(10):**
 - ADL and IADL support
 - Behavioral health therapy options
 - Comorbid condition management
 - Lifestyle changes (eg, nutrition, exercise, tobacco cessation)
 - Medication management
 - No cure
 - Psychotherapy (eg, cognitive behavioral therapy (CBT), individual or group therapy, family therapy)
 - Self-care activities (eg, diet, activity, medication)

- Symptom management
- **EDUCATE: Treatment goals(10):**
 - Complication and progression management
 - Educate and support caregivers
 - Functional improvement and maintenance (eg, ADL, IADL)
 - Psychosocial functioning improved and maintained
 - Quality of life improvement
 - Recurrence prevention
 - Relapse reduction
 - Safety (for self and others)
 - Symptom frequency and severity reduced
- **EDUCATE: Resources:**
 - MedlinePlus, a service of the U.S. National Library of Medicine and National Institutes of Health: www.medlineplus.gov
 - National Alliance on Mental Illness: www.nami.org
 - National Institute of Mental Health: www.nimh.nih.gov
 - National Suicide Prevention Lifeline: www.988lifeline.org
 - Smokefree. National Institutes of Health: www.smokefree.gov
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Psychiatrist referral, if indicated
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Social worker referral, as needed(11)
- **COORDINATE:** Care team meeting, if needed(12)(13)
- **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
- **COORDINATE:** Follow-up to evaluate bipolar disorder understanding, referral status, and need for further assistance(16)(17)
- **SEND:** Your care plan - Bipolar disorder  (Spanish)
- **SEND:** What you need to know - Bipolar disorder  (Spanish)
- **SEND:** When to get help - Bipolar disorder  (Spanish)
- **SEND:** Daily care for your condition - Bipolar disorder  (Spanish)
- **SEND:** Be ready for your doctor visit - Bipolar disorder  (Spanish)
- **SEND:** Be ready with questions for your doctor - Bipolar disorder  (Spanish)
- **SEND:** Tips for good mental health  (Spanish)

P2. Bipolar disorder self-care needed

- **Goal:** Bipolar disorder self-care plan developed, implemented, and maintained(5)(18)(19)[N](#)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Obtaining a medical alert bracelet or necklace
 - **ASSIST:** Appointment scheduling
 - **ASSIST:** Self-care plan creation between patient and provider (eg, goals, management, treating symptoms, getting help)
 - **ASSIST:** Communication between multiple providers(20)
 - **EDUCATE: Self-care plan information(6)(10):**
 - Appointment scheduling and attendance
 - Adherence to plan necessary to achieve goals, and complications of nonadherence (eg, exacerbations, progression, comorbidities)(8)
 - Medication purpose, use, effects, and considerations (eg, timing before meals or exercise)
 - Set small goals every day (eg, get out of bed at a certain time, finish one chore or project, or do something that brings pleasure like listen to music).
 - Health checks, diary, and action plans (eg, write down thoughts, feelings, daily activities, eating times, and when medication taken)
 - Lifestyle change plan (eg, psychotherapy, regular exercise, good sleep hygiene)(8)(21)
 - Symptom management
 - Relapse signs (eg, insomnia, decreased appetite, poor personal hygiene)
 - Behavioral health monitoring and care (eg, depression monitoring and management)
 - Stress management (eg, relaxation and coping techniques)

- Sleep hygiene (eg, regular pattern, calm sleep environment)
- Behavioral health treatment (eg, cognitive behavioral therapy (CBT), individual therapy, family therapy)[D]
- Caregiver support plan, as needed(9)(22)
- Crisis care plan: turn off distractions (eg, TV, radio), seek emergent care, call hotline, call police if escalates; caregivers advised to not argue(23)
- Safety awareness and management (eg, remove suicide risks, de-escalation techniques)(10)(24)
- Support group participation
- Comorbid condition management
- Diet considerations (eg, limit caffeine and alcohol)
- Exercise program within provider's recommendations(25)[N]
- Medical alert bracelet/necklace, obtain and wear
- Laboratory monitoring, as needed
- Wellness checkups (eg, vaccinations, screenings)
- Tobacco cessation plan(26)[N]
- **EDUCATE: When to get emergency help(9):**
 - Suicidal or homicidal actions or plans(10)(24)
 - Crisis plan not working
 - Psychotic behavior (eg, threatening)
 - Hallucinations or delusions, increased or new(7)(8)
 - Seizures, new or complicated (eg, seizure longer than 5 minutes, multiple seizures with loss of consciousness, head injury during seizure)
 - Heart attack (eg, persistent chest pain; pain radiating to the arms, shoulders, neck, back, and/or jaw; cool clammy skin)(27)
- **EDUCATE: When to call the provider for behavior changes(9)(10):**
 - Hallucinations or delusions increase.
 - Self-harm (eg, abuse, mutilation)
 - Insomnia
 - Mood instability (eg, extreme or rapid changes)
 - Self-care changes (eg, ADL or IADL problems)
 - Self-care plan problems (eg, unable to follow, condition exacerbation)
 - Substance misuse (eg, increased use, use interferes with ADL/IADL)(8)
 - Appointments missed (eg, support group, provider)
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)
 - Disorganization of thoughts (eg, flight of ideas, rapidly switching subjects in conversation)
 - Medication problems (eg, side effects, access, administration)(7)
- **EDUCATE: When to call the provider(26):**
 - Comorbid condition exacerbation
 - Constipation
 - Medications newly ordered from other providers
 - Pregnant or planning to get pregnant(26)
 - Weight gain or loss without reason
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Crisis services referral, as needed
- **COORDINATE:** Lifestyle management program referral, as needed(7)
- **COORDINATE:** Social worker referral, as needed(11)
- **COORDINATE:** Support group referral, as needed(28)
- **COORDINATE:** Telehealth referral, if indicated[E](28)
- **COORDINATE:** Vocational rehabilitation referral, as needed(10)(29)[N]
- **COORDINATE:** Tobacco use treatment, as needed
- **COORDINATE:** Care team meeting, if needed(12)(13)
- **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
- **COORDINATE:** Follow-up contact with patient to evaluate understanding and adherence to self-care plan, referral status, and need for further assistance(16)(17)
- **SEND:** Your care plan - Bipolar disorder  (Spanish)
- **SEND:** What you need to know - Bipolar disorder  (Spanish)
- **SEND:** When to get help - Bipolar disorder  (Spanish)
- **SEND:** Daily care for your condition - Bipolar disorder  (Spanish)
- **SEND:** Be ready for your doctor visit - Bipolar disorder  (Spanish)
- **SEND:** Be ready with questions for your doctor - Bipolar disorder  (Spanish)
- **SEND:** Caregiver tips  (Spanish)

- **SEND:** Choosing a place to live 📄 (Spanish)
- **SEND:** Help with sleeping or insomnia 📄 (Spanish)
- **SEND:** How to stop smoking 📄 (Spanish)
- **SEND:** Tips for good mental health 📄 (Spanish)
- **SEND:** Symptoms diary (Illustrated) 📄 (Spanish)
- **SEND:** Safety in the home - Memory loss 📄 (Spanish)
- **Goal:** Behavioral health therapy initiated and maintained
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for information on condition, treatment options, and outcomes
 - **ASSIST:** Identifying coping mechanisms
 - **EDUCATE:** Make and keep all appointments with psychiatrist, psychologist, therapist, or social worker to help address symptoms and provide a supervised program.(22)
 - **EDUCATE:** Guided adjustment of bipolar disorder self-care plan as indicated by provider. Treatment options:
 - Psychotherapy (eg, cognitive behavioral therapy (CBT), dialectic behavior therapy, social rhythm therapy)[F](10)
 - Medication management
 - **EDUCATE:** The effects of substance misuse on behavioral health
 - **COORDINATE:** Behavioral health services referral, if indicated[G]
 - **COORDINATE:** Psychiatrist referral, if indicated
 - **COORDINATE:** Social worker referral, as needed(11)(28)
 - **COORDINATE:** Support group referral, as needed(28)
 - **COORDINATE:** Treatment program referral, if indicated
 - **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
 - **COORDINATE:** Care team meeting, if needed(12)(13)
 - **COORDINATE:** Follow-up contact to review understanding of bipolar disorder treatment options and need for further assistance(16)(17)
 - **SEND:** Daily care for your condition - Bipolar disorder 📄 (Spanish)
 - **SEND:** Be ready for your doctor visit - Bipolar disorder 📄 (Spanish)
 - **SEND:** Be ready with questions for your doctor - Bipolar disorder 📄 (Spanish)
 - **SEND:** Be ready for your doctor visit - Substance-related disorders 📄 (Spanish)
 - **SEND:** Be ready with questions for your doctor - Substance-related disorders 📄 (Spanish)

P3. Bipolar disorder potential complications and comorbidities monitoring needed

- **Goal:** Bipolar disorder potential complications and comorbidities evaluated and addressed(5)[N]
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of problems, comorbidities, and self-care plan[H]:
 - Recently hospitalized patient's evaluation within 1 week of discharge[N]
 - Recently hospitalized patient's evaluation with outpatient healthcare providers (eg, nurse, social worker) within 1 to 3 days of discharge(16)
 - Appointment every 3 to 6 months when patient is symptom free
 - Appointment once a month or more for patient with questionable adherence to treatment plan
 - **EDUCATE:** When and how to get help for worsening bipolar disorder problems
 - **EDUCATE:** Treatment and monitoring for comorbid conditions (eg, coronary artery disease, high blood pressure, heart valve disease)(9)(30)
 - **EDUCATE:** Follow-up appointments for assessment of clinical status[I]
 - **EDUCATE:** Treating bipolar disorder can help improve outcomes of coexisting chronic conditions.
 - **EDUCATE:** Certain medications prescribed for bipolar disorder may worsen comorbid conditions.(7)
 - **EDUCATE:** Effects of behavioral health condition and conjunctive medication usage during pregnancy and lactation(31)
 - **COORDINATE:** Specialist referral, if indicated
 - **COORDINATE:** Treatment program referral, if indicated(32)
 - **COORDINATE:** Preconception specialist referral, as needed
 - **COORDINATE:** Care team meeting, if needed(12)(13)

- **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
- **COORDINATE:** Follow-up contact to review self-care plan and need for further assistance(16)(17)
- **SEND:** Be ready for your doctor visit - Bipolar disorder  (Spanish)
- **SEND:** Be ready with questions for your doctor - Bipolar disorder  (Spanish)
- **SEND:** Narcotic medicine side effects  (Spanish)
- **SEND:** Pain medicines - Addiction and withdrawal  (Spanish)
- **SEND:** Be ready for your doctor visit - Substance-related disorders  (Spanish)
- **SEND:** Be ready with questions for your doctor - Substance-related disorders  (Spanish)

P4. Bipolar disorder medication evaluation needed

- **Goal:** Bipolar disorder medication evaluation completed and medication regimen initiated, if indicated
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for evaluation of condition status
 - **EDUCATE:** When and how to get help for worsening bipolar disorder problems
 - **EDUCATE:** Medication management needs common for bipolar disorder^[J](10)
 - **COORDINATE:** Follow-up after provider contact for evaluation of medication changes(16)(17)

P5. Bipolar disorder medication management needed

- **Goal:** Medication information understood(33)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for discussion on appropriate medication
 - **ASSIST:** Providing reconciled medication list to all providers. See Medication Reconciliation Tool  SR for more information.
 - **EDUCATE: Purpose:** Importance of understanding what each medication is for, how it should be taken, and what to do if dose is missed
 - **EDUCATE:** Medication indications, side effects, and considerations frequently change. See a drug reference for specific information not listed here.
 - **EDUCATE:** Administration techniques:
 - Route (eg, oral, intramuscular (IM), intravenous (IV))
 - With or without foods or liquids (eg, milk)
 - Timing of administration
 - Appropriate measuring device (eg, liquid medications)
 - Proper storage
 - **EDUCATE: Get emergency help:** Adverse or allergic reactions (eg, wheezing, chest tightness, fever, itching, bad cough, blue skin color, hemorrhage, seizures, swelling of face, lips, tongue, or throat)
 - **EDUCATE: Side effects:** Management, and when to contact provider
 - Common side effects include gastrointestinal changes, dry mouth, fatigue, insomnia, rash, dizziness, and headache.
 - **EDUCATE: Considerations:**
 - Discuss medication with provider if pregnant, planning a pregnancy, or breast-feeding.
 - Comorbidities may be contraindications to the use of a drug (eg, liver disease, diabetes).
 - Health checks, diary, and action plans (eg, blood pressure, weight, heart rate)
 - Safety measures (eg, sun protection, driving, or operating machinery)
 - Contact provider or pharmacist prior to taking any other over-the-counter or prescribed medication.
 - **COORDINATE:** Clinical monitoring appropriate to medication use (eg, laboratory tests)
 - **COORDINATE:** Communication between patient and pharmacy (for obtaining prescriptions, setting up home medication delivery, review for drug-drug interactions)^[N]
 - **COORDINATE:** Communication between care team members and medication list reconciliation
 - **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, and identify need for further assistance(16)(17)
 - **COORDINATE:** Laboratory testing, if indicated
 - **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
 - **COORDINATE:** Therapeutic drug level monitoring

- **SEND:** Managing your medicines (Illustrated)  (Spanish)

P6. Anticonvulsant medication management needed

- **Goal:** Anticonvulsant medication initiated and maintained(6)(34)
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Anticonvulsants may be used as mood stabilizers or for seizure treatment.
 - **EDUCATE: Call provider immediately:** Vision problems, burning or tingling sensation, confusion, slowing of mental and physical ability, memory problems, speech problems, unusual fatigue, bleeding problems
 - **EDUCATE: Side effects:** Appetite changes, tremors
 - **EDUCATE: Considerations:**
 - Obtain baseline platelet count and bleeding time before starting valproic acid.
 - Valproic acid is not advisable for women of childbearing age due to the risk of birth defects.
 - **SEND:** Anticonvulsant medicines  (Spanish)

P7. Antidepressant medication management needed

- **Goal:** Antidepressant medication initiated and maintained(6)(35)
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Antidepressants may be indicated to help relieve symptoms of depression.
 - Effects may take 2 to 8 weeks.
 - Most people need to be on medication at least 18 to 24 months after adequate response to symptoms.
 - **EDUCATE: Call provider immediately:** Suicidal ideation
 - **EDUCATE: Side effects:** Sedation, dry mouth
 - **EDUCATE: Considerations:**
 - Augmentation agent may be added or change in antidepressant may occur if symptoms do not improve.
 - Withdrawal symptoms may occur when discontinued.
 - **SEND:** Antidepressant medicines  (Spanish)

P8. Atypical antipsychotic medication management needed

- **Goal:** Atypical antipsychotic medication initiated and maintained
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Atypical antipsychotics may be indicated for rapid control of acute mania to decrease agitation, aggression, and dangerous behavior.(36)(37)
 - Second-generation antipsychotic medication
 - **EDUCATE: Call provider immediately:** Neuroleptic malignant syndrome (NMS) (eg, extreme muscle rigidity, high fever, tachycardia, tachypnea, diaphoresis, stupor, fluctuations in blood pressure)[K](38)
 - **EDUCATE: Side effects:** Difficulty urinating; jaundice; shuffling walk; fainting spells; seizures; pharyngitis; muscle spasms; excitement or restlessness; jerky movements of head, face, or neck; tachycardia; weight gain; blurred vision(39)
 - Increased risk of metabolic side effects, including obesity, diabetes, and dyslipidemia(37)
 - **EDUCATE: Considerations(37):**
 - Use cautiously with history of seizures, liver disease, heart disease, blood disorders, or pregnancy.(31)
 - Monitor blood glucose, cholesterol, and triglyceride levels.
 - Talk to provider before discontinuing medication.
 - **COORDINATE:** Electrocardiogram (ECG) monitoring, if indicated(37)
 - **COORDINATE:** Laboratory testing

P9. Mood stabilizer medication management needed

- **Goal:** Mood stabilizer medication initiated and maintained(36)
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Mood stabilizers may be indicated for acute mania and maintenance therapy.
 - **EDUCATE: Call provider immediately:** Rash, fever, jaundice, urine output changes, dysrhythmia(40)
 - Lithium toxicity (eg, tremor, confusion, persistent vomiting or diarrhea, excessive urine output)(7)
 - Divalproex sodium (eg, ataxia, vomiting, loss of appetite, hair loss, fatigue)
 - **EDUCATE: Side effects:** Drowsiness, dry mouth, hypotension, arrhythmias, polyuria, weight gain(38)
 - Carbamazepine may cause blood dyscrasias or enzyme induction (which may lower plasma levels of other medications).
 - **EDUCATE: Considerations(31):**
 - Avoid in patients with cardiac or renal disease, brain damage, dehydration, pregnancy, and lactation.
 - Take vital signs 2 to 3 times a day if cardiac side effects present. Dosage may need adjusting.

- Use cautiously with thyroid disease, diabetes, urinary retention, history of seizures, and with the elderly.
- **COORDINATE:** Therapeutic drug level monitoring
- **SEND:** How to check your blood pressure (Illustrated)  (Spanish)
- **SEND:** Blood pressure diary (Illustrated)  (Spanish)
- **SEND:** How to measure your heart rate (Illustrated)  (Spanish)

P10. Sleeping problems present

- **Goal:** Sleep improved 
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Scheduling provider appointment for medical evaluation of sleep changes and creation of self-care plan to include(41)(42):
 - Identifying techniques to improve sleep quality(43)
 - Medication changes that can help patient sleep better (eg, changing the time stimulants are taken to early in the day or not taking diuretic before bedtime)
 - Review of herbal preparations that may promote sleep(44)
 - Creation of sleep diary(45)
 - **EDUCATE:** Sleep hygiene techniques(41)(43)(46):
 - Schedule sleep (go to bed and get up at same time every day).
 - Spend no more than 8 hours per day in bed. Amount of average sleep time for adult is 7 to 8 hours.
 - Limit napping to once a day for 20 to 30 minutes in early afternoon.
 - Sleep in a dark, quiet bedroom.
 - Use bedroom only for sleeping, not to watch TV, work, use electronics, or do other activities.
 - Avoid pets in the bed.
 - Get safe amount of sunlight every day and avoid bright lights before bedtime.
 - Cut down on drinking fluids in the afternoon if nighttime urination is a problem.
 - Avoid substances that may influence sleep (eg, caffeine, stimulants, alcohol).
 - Stop smoking; nicotine is a stimulant.
 - Get regular exercise, but avoid vigorous exercise 2 hours before bedtime.(47)
 - Write down urgent thoughts if mind-racing is a problem.
 - Do not eat big meals before bedtime; gastric process can cause sleep problems.
 - **EDUCATE:** Herbal preparations to help sleep (eg, melatonin, tryptophan, valerian), as approved by provider(48) 
 - **EDUCATE:** Behavioral health treatment options and benefits (eg, cognitive behavioral therapy for insomnia)(45)(49) 
 - **EDUCATE:** Hypnotic medications use and misuse, if prescribed for insomnia(42)
 - **COORDINATE:** Sleep specialist referral, if indicated(48)
 - **COORDINATE:** Behavioral health services referral, if indicated(45)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
 - **COORDINATE:** Follow-up contact to evaluate sleep, status of referrals, and need for further assistance(16)(17)
 - **SEND:** Help with sleeping or insomnia  (Spanish)

P11. Home safety management information needed

- **Goal:** Home safety management plan understood and followed(50)
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of dizziness, unexplained falls, memory loss, medication issues, or other home safety concerns 
 - **ASSIST:** Identifying patient's needs, abilities, preferences, and risks for problems. See Home Safety Assessment  SR for further information.[L](51)
 - **ASSIST:** Medication list and pharmacy review(41)
 - **ASSIST:** Identifying environmental structural problems or modifications (eg, steps, lighting, access to bathroom or kitchen)(52)
 - **ASSIST:** Obtaining emergency medical identification, if needed
 - **ASSIST:** Acquiring and using devices for communication (eg, cell phone, alarm system) in case of emergency(53)
 - **ASSIST:** Identifying and moving to different living arrangements, as needed
 - **ASSIST:** Creating emergency evacuation plan, as needed

- **EDUCATE:** Home changes to help prevent falls:
 - Remove all throw rugs and clutter from floors.
 - Repair carpet tears, broken tiles, or hardwood floor ruts that could cause tripping.
 - Clear walking areas of electrical cords or wires.
 - Move furniture to allow for use of crutches or walker.
 - Keep all areas well lit.(54)
 - Clean up spills immediately.
- **EDUCATE:** Self-care safety activities(55)(56):
 - Rise slowly from lying or sitting position.
 - Wear glasses or contact lenses, if needed.
 - Use handrails, grab bars, or adaptive devices as needed.
 - Wear shoes with nonslip soles.
 - Use no-slip mats in shower or tub.
 - Pet-related falls mitigation (eg, not chasing pet, obedience training)
 - Know which medication may cause problems (eg, dizziness, weakness, or balance problems).
 - Exercise program within provider's recommendations(25)(52)[N](#)
- **EDUCATE:** Safety hazard removal and ways to cut down safety risks(56):
 - Architectural changes that can improve safety (eg, widening doors to allow for wheelchair, putting grab bars in bathroom)
 - Hot water tank temperature setting between 110 to 120 degrees F (43.4 to 48.9 degrees C)(57)
 - Snow and ice removal plan
 - Chairs and other seats stable and at height to facilitate easy rising
 - Electrical appliances, cords, and outlets safe, used appropriately, and in good repair
- **EDUCATE:** Fire prevention:
 - Smoke detector functioning, and plan to change battery regularly
 - Fireplace in good condition, with screen and chimney cleaned regularly
 - Smoking habits as safe as possible
 - Electrical and heating units away from flammable material
 - Electrical appliances, cords, and outlets safe, used appropriately, and in good repair
 - Evacuation plan in event of fire that includes location of available exits
 - How to put out flames if clothes catch on fire
- **EDUCATE:** When to get emergency help:
 - Fall complications (eg, head injury, broken bone, significant ADL changes)
 - Fear of personal harm from someone (eg, spouse, parent). See Intimate Partner Violence [CCG](#).
 - Stroke, heart attack, or blood clot
- **EDUCATE:** When to call the provider:
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)
 - Depression (eg, feeling sad, hopeless, overwhelmed)(6)
 - Falls
 - Medication problems (eg, side effects, access, administration)
 - Self-care changes (eg, ADL or IADL problems)
 - Physical activity level declines (eg, fatigue, weakness).
- **COORDINATE:** Pharmacy medication support, if indicated[M](58)
- **COORDINATE:** Specialist referral, if indicated
- **COORDINATE:** Home care referral, if indicated(59)
- **COORDINATE:** Rehabilitation therapy referral, if indicated(52)(59)
- **COORDINATE:** Medical equipment and supplies, if indicated(60)
- **COORDINATE:** Community services referral, as needed[N]
- **COORDINATE:** Social worker referral, as needed(11)
- **COORDINATE:** Protective services or legal authorities, as needed(58)
- **COORDINATE:** Emergency plans for regional weather or other emergencies (eg, earthquakes, floods, tornadoes)
- **COORDINATE:** Follow-up contact to evaluate safety, referral status, and need for further assistance(16)(17)
- **SEND:** Choosing a place to live  (Spanish)
- **SEND:** Exercising to improve balance (Illustrated)  (Spanish)
- **SEND:** Exercising when you have neurologic problems  (Spanish)
- **SEND:** How to get out of bed safely (Illustrated)  (Spanish)
- **SEND:** How to move safely in bed (Illustrated)  (Spanish)
- **SEND:** How to use a cane (Illustrated)  (Spanish)
- **SEND:** How to use a walker (Illustrated)  (Spanish)

- **SEND:** Managing your medicines (Illustrated)  (Spanish)
- **SEND:** Safe lifting (Illustrated)  (Spanish)
- **SEND:** Safety in the home - Fall protection  (Spanish)
- **SEND:** Safety in the home - Fire and other risks  (Spanish)
- **SEND:** Safety in the home - Memory loss  (Spanish)
- **SEND:** Walking with crutches (Illustrated)  (Spanish)
- **SEND:** Ways to save energy  (Spanish)

P12. Home safety issues not managed

- **Goal:** Safety issues identified and addressed(61)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of dizziness, unexplained falls, memory loss, medication issues, or other home safety concerns
 - **ASSIST:** Contact with legal authorities, as appropriate
 - **EDUCATE:** When and how to get help for worsening problems or concerns for safety
 - **COORDINATE:** Social worker referral, as needed(11)
 - **COORDINATE:** Medical equipment and supplies, if indicated(60)
 - **COORDINATE:** Protective services or legal authorities, as needed
 - **COORDINATE:** Pharmacy medication support, if indicated(58)
 - **COORDINATE:** Rehabilitation therapy referral, if indicated(52)(59)
 - **COORDINATE:** Specialist referral for comorbidities, if indicated
 - **COORDINATE:** Audiologist referral, if indicated
 - **COORDINATE:** Ophthalmologist referral, if indicated
 - **COORDINATE:** Home care referral, if indicated(59)
 - **COORDINATE:** Follow-up contact to evaluate safety issues, referral status, and need for further assistance(16)(17)

P13. Healthy diet information needed

- **Goal:** Healthy diet better understood(62)(63) 
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for dietary review and self-care plan
 - **EDUCATE:** Healthy body weight and body mass index (BMI) for patient(64):
 - BMI equals body weight in kilograms divided by the square of the height in meters (kg/m²).
 - BMI of 18.5 to 24.9 is a normal weight.
 - BMI of 25 to 29.9 is overweight.
 - BMI of 30 or greater is obese.
 - **EDUCATE:** Healthy diet general recommendations(65)(66):
 - Well-balanced diet (ie, high in fruits and vegetables, low in sodium, fats, and sugars)
 - Drink enough fluids.
 - Safe eating habits (eg, do not skip meals)
 - Calorie amount appropriate (eg, factors: age, gender, activity level)
 - **EDUCATE:** Adequate fluid intake^[O](66)(67):
 - Amount of fluid intake needed depends on patient attributes, including medical condition, level of physical activity, and exposure to heat. 
 - Men age 19 years and older: 15.6 cups (3.7 L) per day from all sources (eg, food and fluids)
 - Women age 19 years and older: 11.4 cups (2.7 L) per day from all sources (eg, food and fluids)
 - **COORDINATE:** Dietitian referral, if indicated(68)
 - **COORDINATE:** Community services referral, as needed
 - **COORDINATE:** Follow-up contact with patient to evaluate diet understanding and adherence, referral status, and need for further assistance(16)(17)
 - **SEND:** Healthy eating (Illustrated)  (Spanish)
 - **SEND:** How to read food labels (Illustrated)  (Spanish)
 - **SEND:** Weight diary (Illustrated)  (Spanish)

- **SEND:** Meal diary (Illustrated)  (Spanish)

P14. Potential nutritional problems

- **Goal:** Nutritional problems resolved(63)(69)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for dietary review and self-care plan
 - **ASSIST:** Creation of nutrition diary
 - **COORDINATE:** Laboratory testing, if indicated
 - **COORDINATE:** Calorie or fluid counting
 - **EDUCATE:** Nutritional recommendations for patient's health status(66)
 - **EDUCATE:** Food and drug interactions potentially impacting nutritional status
 - **COORDINATE:** Dietitian referral, if indicated(68)
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Pharmacy medication support, if indicated
 - **COORDINATE:** Rehabilitation therapy referral, if indicated
 - **COORDINATE:** Follow-up contact with patient to evaluate nutritional status, referral status, and need for further assistance(16)(17)
 - **SEND:** Eating to keep up your energy  (Spanish)
 - **SEND:** Eating when your stomach is upset  (Spanish)
 - **SEND:** Mouth sores or infections  (Spanish)
 - **SEND:** How to read food labels (Illustrated)  (Spanish)
 - **SEND:** Meal diary (Illustrated)  (Spanish)

P15. Food label reading information needed

- **Goal:** Food label understanding improved
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider, nurse, or dietitian to provide in-person instruction on how to read food labels
 - **EDUCATE:** Food labels contain the nutritional value of the product(70)(71):
 - Percent of daily values is usually based on a 2000-calorie (8374-kJ) diet.
 - Nutritional values are listed per one serving.
 - A package may have more than one serving in the whole package.
 - Nutritional value should be doubled, tripled, and so on based on the number of servings eaten.
 - Serving size varies from product to product.
 - Form of food has impact on nutrition (eg, increased sugar content of orange juice vs one orange).
 - **COORDINATE:** Dietitian referral, if indicated
 - **COORDINATE:** Follow-up contact to evaluate food label understanding, referral status, and need for further assistance(16)(17)
 - **SEND:** How to read food labels (Illustrated)  (Spanish)

P16. Social support needed

- **Goal:** Social support improved(72)(73) 
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Patient in establishment of a helpful social network(74):
 - Identification of individuals in support network
 - Engaging in social support system (gathering information, helping patient develop skills, encouraging shifts in perspective, and allowing patient to vent about issues)
 - **EDUCATE:** Ways to access social support
 - **EDUCATE:** Role of social support in quality of life (eg, improved self-image, coping)(73)

- **EDUCATE:** Patient's role in social support relationships (eg, be aware of how actions impact the feelings of family and friends)(75)
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Community services referral, as needed(76)
- **COORDINATE:** Social worker referral, as needed(11)
- **COORDINATE:** Support group referral, as needed(58)(77)
- **COORDINATE:** Support network contact (eg, church, social groups), as needed(74)
- **COORDINATE:** Follow-up to evaluate social support, referral status, and need for further assistance(16)(17)

P17. Potential hazardous or at-risk alcohol use present

- **Goal:** Alcohol effects and complications of misuse understood(78)
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of effects of alcohol use and self-care plan, if needed
 - **EDUCATE:** Health effects of drinking(78)(79):
 - Light to moderate intake (2 drinks per day for men and 1 drink per day for women) may be linked with poor health outcomes.[C](80)(81)
 - Heavy drinking can cause acute alcohol poisoning and nutritional deficiencies.
 - Long-term effects of alcohol misuse include cirrhosis, pancreatitis, peripheral neuropathy, and cancer.(82)
 - **EDUCATE:** Follow drinking limits set by provider.
 - **EDUCATE:** Comparison of personal drinking to national norms(79):
 - Safe drinking levels for men (younger than 65 years) are 14 drinks or fewer per week and 4 drinks or fewer on one occasion.
 - Safe drinking levels for women are 7 drinks or fewer per week and 3 drinks or fewer on one occasion.
 - Safe drinking levels for healthy adults age 65 years are 7 drinks or fewer per week and 3 drinks or fewer on one occasion.
 - No drinking during pregnancy is safe. Caution should be used when breast-feeding; consumption of a single drink may be safe if the mother waits 4 hours before breast-feeding or expresses breast milk for feeding prior to consuming the alcoholic drink.
 - **EDUCATE:** Harmful alcohol interactions with medications (eg, with analgesics, antibiotics, anticoagulants, antidepressants, antihistamines, anticonvulsants, medications for diabetes, beta-blockers, and sedative/hypnotic medications)
 - **EDUCATE:** Role of self-help programs (eg, Alcoholics Anonymous) and cognitive behavioral therapy(83)
 - **EDUCATE: When to get emergency help(84):**
 - Drug overdose or alcohol poisoning (eg, loss of consciousness, problems breathing, coma, low blood pressure or body temperature)
 - Delirium tremens (DT), usually starts 48 to 72 hours after last drink (eg, confusion, disorientation, tachycardia, severe tremors, hallucinations, and seizures)(85)
 - **EDUCATE: When to call the provider for behavior changes:**
 - Substance misuse (eg, increased use, use interferes with ADL/IADL)
 - Medication problems (eg, side effects, access, administration)
 - Depression (eg, feeling sad, hopeless, overwhelmed)(6)(35)
 - **EDUCATE: Resources:**
 - Al-Anon Family Groups: www.al-anon.org
 - Alcoholics Anonymous World Services: www.aa.org
 - National Institute on Alcohol Abuse and Alcoholism: www.niaaa.nih.gov
 - Substance Abuse and Mental Health Services Administration: www.samhsa.gov
 - **COORDINATE:** Behavioral health services referral, if indicated
 - **COORDINATE:** Community services referral, as needed(83)
 - **COORDINATE:** Support group referral, as needed
 - **COORDINATE:** Social worker referral, as needed(11)
 - **COORDINATE:** **Substance-related disorders** assessment, if indicated(83)
 - **COORDINATE:** Care team meeting, if needed(12)(13)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
 - **COORDINATE:** Follow-up contact to evaluate alcohol use status, referral status, and need for further assistance(16)(17)
 - **SEND:** Tips for good mental health  (Spanish)
 - **SEND:** What you need to know - Substance-related disorders  (Spanish)
 - **SEND:** Be ready for your doctor visit - Substance-related disorders  (Spanish)

- **SEND:** Be ready with questions for your doctor - Substance-related disorders  (Spanish)

P18. Substance misuse information needed

- **Goal:** Substance misuse information understood(82)(86)(87)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of substances used and impact on self-care plan
 - **EDUCATE:** Common substances that are misused
 - **EDUCATE:** Harm reduction measures (eg, not driving while intoxicated, communicable disease prevention)
 - **EDUCATE:** Risks associated with misusing substances (eg, health risks, relationship changes)
 - **EDUCATE:** Substance misuse or addiction signs (eg, behavior changes, drug seeking)(88)
 - **EDUCATE:** Withdrawal expectations
 - **EDUCATE:** When and how to get help if substances misused
 - **EDUCATE:** Complications if substance use not treated (eg, liver or renal failure)(89)
 - **COORDINATE:** Community services referral, as needed(83)
 - **COORDINATE:** Care team meeting, if needed(12)(13)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
 - **COORDINATE:** Follow-up contact to evaluate substance misuse understanding, referral status, and need for further assistance(16)(17)

P19. Substance use disorder potentially present

- **Goal:** Substance use reduced or stopped and effects managed(82)(88)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of substances used and impact on self-care plan
 - **ASSIST:** Appointment scheduling for behavioral health provider evaluation of new or worsening problems with substance use
 - **ASSIST:** Accessing benefits for substance use disorder programs (for medication or counseling)
 - **EDUCATE: Self-care plan information(84):**
 - Action plan (eg, managing cravings, reacting to triggers)
 - Symptom management (eg, withdrawal symptoms)
 - Daily care (eg, diet, exercise, medication)
 - Support person (eg, sponsor) help
 - Support group participation
 - Behavioral health treatment (eg, cognitive behavioral therapy (CBT), individual therapy, family therapy)(90)
 - Stress management (eg, relaxation and coping techniques)
 - Risk factor management (eg, trigger avoidance)
 - **EDUCATE: When to get emergency help(84):**
 - Drug overdose or alcohol poisoning (eg, loss of consciousness, problems breathing, coma, low blood pressure or body temperature)
 - Suicidal or homicidal actions or plans(91)
 - Delirium tremens (DT), usually starts 48 to 72 hours after last drink (eg, confusion, disorientation, tachycardia, severe tremors, hallucinations, and seizures)(85)
 - Headache (eg, sudden severe, unrelieved by analgesics)
 - Heart attack (eg, persistent chest pain; pain radiating to the arms, shoulders, neck, back, and/or jaw; cool clammy skin)(27)
 - Stroke (eg, face drooping, arm weakness, speech problems, bad headache, vision changes, loss of balance or coordination)(92)
 - **EDUCATE: When to call the provider for behavior changes(84)(89):**
 - Substance misuse (eg, increased use, use interferes with ADL/IADL)
 - Depression (eg, feeling sad, hopeless, overwhelmed)(6)(35)
 - Medication problems (eg, side effects, access, administration)
 - Legal problems (eg, being arrested, stealing, or financial difficulties)
 - **EDUCATE: Resources:**
 - Cocaine Anonymous World Services: www.ca.org
 - Marijuana Anonymous World Services: www.marijuana-anonymous.org
 - Narcotics Anonymous: www.na.org

- Substance Abuse and Mental Health Services Administration: www.samhsa.gov
- **COORDINATE:** **Substance-related disorders** assessment, if indicated(83)
- **COORDINATE:** Support group referral, as needed(84)
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Psychiatrist referral, if indicated(93)
- **COORDINATE:** Treatment program referral, if indicated
- **COORDINATE:** Social worker referral, as needed(11)
- **COORDINATE:** Community services referral, as needed(83)
- **COORDINATE:** Care team meeting, if needed(12)(13)
- **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
- **COORDINATE:** Follow-up contact to evaluate substance discontinuation, referral status, and need for further assistance(16)(17)
- **SEND:** Pain medicines - Addiction and withdrawal  (Spanish)
- **SEND:** When to get help - Substance-related disorders  (Spanish)
- **SEND:** Be ready for your doctor visit - Substance-related disorders  (Spanish)
- **SEND:** Be ready with questions for your doctor - Substance-related disorders  (Spanish)
- **SEND:** Tips for good mental health  (Spanish)

P20. Tobacco use present

- **Goal:** Tobacco abstinence achieved(94) **NNNN**
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for evaluation for nicotine replacement therapy (NRT) or other medications to help patient quit using tobacco
 - **ASSIST:** Developing a plan to quit(95):
 - Setting quit date(96)
 - Identifying reasons to quit (eg, add years to your life, for someone you love, save money, feel better)
 - Identifying techniques to quit (ie, "cold turkey," using nicotine replacement, or gradual withdrawal)(97)
 - Identifying coping mechanisms
 - **ASSIST:** Identifying resources to help tobacco cessation
 - **EDUCATE:** Impact of tobacco on life(82):
 - Current symptoms caused by tobacco use
 - Costs of tobacco use
 - Impact of tobacco use on home environment
 - Risks of continued smoking on health (eg, cancer, stroke, gum disease)
 - **EDUCATE:** Benefits of tobacco cessation:
 - Within minutes to days - lower blood pressure, lower carbon monoxide levels, better stamina, and improved smell and taste
 - Within 2 to 4 weeks - less risk of respiratory infection
 - Within 1 to 3 months - improved lung function and circulation
 - Within 1 year - 50% reduction in risk for myocardial infarction(98)
 - Within 5 to 15 years - cardiovascular risk equal to that of someone who has never smoked
 - Within 10 years - lung cancer risk cut in half
 - **EDUCATE:** Ways to stop smoking or using tobacco(82):
 - Get support (support group, counseling, family buy-in, written self-help materials).(99) **NNNNN**
 - Talk to providers for help.
 - Stress management (eg, relaxation and coping techniques)(96)
 - Nicotine replacement therapy (eg, patch, gum) or medications that may help **P** **N**
 - Healthy lifestyle (good nutrition, activity)(96)(100) **N**
 - **EDUCATE:** Withdrawal symptoms that may be experienced (eg, trouble sleeping, replacing smoking urge with eating)
 - **EDUCATE:** **Resources:**
 - American Lung Association: www.lung.org
 - Centers for Disease Control and Prevention: www.cdc.gov/tobacco
 - Smokefree. National Institutes of Health: www.smokefree.gov
 - **COORDINATE:** Motivational counseling if patient not ready to quit(101)
 - **COORDINATE:** Tobacco use treatment, as needed
 - **COORDINATE:** Participation in stop-smoking programs (eg, American Cancer Society, American Lung Association)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)

- **COORDINATE:** Follow-up contact to monitor for withdrawal and evaluate tobacco use, referral status, and need for further assistance(16)(17)
- **SEND:** How to stop smoking 🇪🇸 (Spanish)
- **Goal:** Tobacco cessation medication treatment initiated and maintained(40)NN
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Medication is used to reduce withdrawal feeling during smoking cessation.
 - Bupropion sustained-release and varenicline are first-line treatment, either alone or in combination.
 - Bupropion is recommended for 7 to 12 weeks. Continued treatment may be needed.
 - Varenicline can be used as maintenance therapy to prevent relapse.(102)
 - **EDUCATE: Call provider immediately:** Suicidal ideation, behavioral changes, or seizures(103)
 - **EDUCATE: Side effects:** Insomnia, tachycardia
 - **EDUCATE: Considerations:** Avoid bupropion in patients with seizure disorders.(40)
 - **SEND:** How to stop smoking 🇪🇸 (Spanish)
- **Goal:** Nicotine replacement therapy (NRT) initiated and maintained(40)
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** NRT is used to reduce withdrawal feeling during smoking cessation.
 - Dosage is individualized based on need and response of patient.
 - If using patch, apply one new patch every 24 hours on skin that is dry, clean, and hairless. Rotate skin sites. Do not cut patch, and do not use more than one patch at a time.
 - If using lozenge or gum, do not eat or drink for 15 minutes before or while utilizing.
 - **EDUCATE: Call provider immediately:** Nicotine toxicity (eg, nausea, abdominal pain, vomiting, diarrhea, sweating, disturbed vision or hearing, palpitations, altered respirations)
 - **EDUCATE: Side effects:** Anxiety, sleep disorders, depression, myalgia
 - Patch may cause redness or swelling of the skin around the patch that does not go away.
 - **EDUCATE: Considerations:** Avoid nicotine use in immediate post-MI period or in presence of serious arrhythmias or severe or worsening angina.
 - **SEND:** How to stop smoking 🇪🇸 (Spanish)

P21. Tobacco use in patient history

- **Goal:** Tobacco abstinence maintained(82)(104)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for evaluation of improved or stabilized health status, for reinforcement of abstinence from tobacco
 - **EDUCATE:** How to keep environment tobacco free
 - **EDUCATE:** Encourage continued abstinence from smoking and review tobacco cessation strategies.
 - **EDUCATE: Resources:**
 - American Lung Association: www.lung.org
 - Centers for Disease Control and Prevention: www.cdc.gov/tobacco
 - Smokefree. National Institutes of Health: www.smokefree.gov
 - **COORDINATE:** Tobacco use treatment, as needed
 - **COORDINATE:** Support group referral, as needed
 - **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
 - **COORDINATE:** Follow-up contact to evaluate tobacco use, referral status, and need for further assistance(16)(17)

P22. Vaccinations not up to date, increasing infection risk

- **Goal:** Vaccinations up to date(105)(106)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **ASSIST:** Identifying vaccines patient may need that are not contraindicated based on clinical status
 - **EDUCATE:** Purpose and benefits of vaccinations
 - **EDUCATE:** Recommended immunizations and timing of administration:
 - Centers for Disease Control and Prevention's vaccination schedule: www.cdc.gov/vaccines(107)

- Vaccine schedules may need to be altered based on medications; check with the specialist provider for information.(108)
- Possible side effects or complications associated with getting vaccinated
- **EDUCATE:** Influenza vaccination annually(109):
 - Side effects are mild and last 1 or 2 days.
 - Common side effects with shot are sore, red, or swollen area where shot was given; fever; and aches.
 - Common side effects with nasal spray are mild and last 1 or 2 days; they include runny nose, headache, sore throat, and cough.
 - Allergic reaction rare and is usually caused by a chicken egg allergy
- **EDUCATE:** Pneumococcal vaccination potential side effects:
 - Side effects are mild and last 1 or 2 days.
 - Common side effects include a sore, red, or swollen area where shot was given; fever; and muscle aches.
 - Serious side effects are rare.
- **EDUCATE:** COVID-19 vaccination potential side effects(107):
 - Side effects are mild and last 1 or 2 days.
 - Common side effects include a sore, red, or swollen area where shot was given; chills; fatigue; and aches.
 - Serious side effects are rare.
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Vaccinations
- **COORDINATE:** Follow-up to evaluate vaccination understanding, referral status, and need for further assistance(16)(17)
- **SEND:** Coronavirus disease 2019 (COVID-19) vaccines  (Spanish)
- **SEND:** Flu vaccine  (Spanish)
- **SEND:** Pneumonia vaccine for adults  (Spanish)
- **SEND:** Hepatitis A vaccine  (Spanish)
- **SEND:** Hepatitis B vaccine  (Spanish)

Evidence Summary

Quality standards: The Case Management Society of America's Standards of Practice for Case Management principles include facilitation of informed decision making, counseling, and health education with patient self-care that is responsive to the patient's preferences (eg, culture, values, needs).(110)

Quality standards: Standards indicate that program content should include an individualized self-care plan that includes patient preferences, prioritized goals, interventions with information on whom to contact in urgent situations, and scheduled follow-up. Programs should facilitate communication and collaboration with practitioners in a timely manner.(110)(111)(112)(113)

National guidelines: The 2018 Physical Activity Guidelines Advisory Committee advises that adults with chronic conditions or disabilities, who are able, should follow the key activity guidelines for adults. The guidelines recommend that adults should do at least 150 to 300 minutes a week of moderate-intensity or 75 to 150 minutes a week of vigorous-intensity aerobic physical activity, or an equivalent combination of moderate-intensity and vigorous-intensity aerobic activity. They should also do muscle-strengthening activities on 2 or more days a week. Older adults should do multicomponent physical activity that includes balance training as well as aerobic and muscle-strengthening activities.(114)

Evidence: A randomized controlled study based on 526 patients found that smoking cessation can be achieved among people with severe mental illness without negatively impacting mental health (eg, depression, anxiety).(115)

National guidelines: The National Institute for Health and Care Excellence (NICE) recommends offering supported employment programs to patients with bipolar disorder in primary or secondary care who wish to find or return to work.(10)

Quality standards: Standards indicate that programs should ensure that during care transitions the care plan is re-evaluated, modified, and communicated to appropriate care team members (eg, providers, caregivers) as needed to support the care transition.(110)(111)(112)(113)

Transition consideration: Several studies demonstrated a reduction in 30-day hospital readmissions when the patient follow-up was within 7 days of hospital discharge.(116)(117)(118) Earlier follow-up (eg, within 48 hours after hospital discharge) may also help reduce hospital readmissions for high-risk or vulnerable patients.(16) Follow-up appointment attendance rates were higher when appointments were scheduled before hospital discharge.(119)(120)

Outcomes: Several studies support the role of the pharmacist in medication reconciliation to identify and resolve pharmacy-related problems and improve outcomes (eg, prevention of medication-related adverse events, discontinuation of inappropriate medication).(121)

Quality standards: Standards indicate that program content should include a condition treatment plan, self-management and self-monitoring, and health behaviors (eg, sleep patterns). Programs should have a process for following up with patients on the status of referrals.(110)

Specialty society: In a systematic review of the pharmacologic treatment of chronic insomnia in adults, the American Academy of Sleep Medicine does not recommend diphenhydramine, melatonin, tryptophan, or valerian for treating sleep onset or maintenance.(44)

Specialty society: The American College of Physicians recommends cognitive behavioral therapy for insomnia as the first-line treatment for patients with chronic insomnia (trouble falling or staying asleep for 3 or more nights a week for at least 3 months, with daytime consequences). Cognitive behavioral therapy for insomnia generally includes cognitive therapy with sleep restriction, relaxation training, and stimulus control for 5 to 8 sessions.(122)

Evidence: Multiple studies indicate that the large number and types of medications consumed by the elderly increase the risk of falls and should be evaluated.(41)(61)(123)

National guidelines: The U.S. Preventive Services Task Force recommends exercise-based interventions to help reduce falls for people 65 years and older. Other interventions (eg, behavioral therapy, nutrition therapy, environmental modification, medication changes) may be considered based on the patient's preferences, the benefit-to-harm balance, and the circumstances of prior falls. Vitamin D supplementation is not recommended for fall prevention for older people without a known deficiency or osteoporosis.(52)

National guidelines: The Institute of Medicine does not specify how much total fluid should come from water. Their recommendation is based on the median of total water intake on dietary surveys, which include water from liquids and solids. For healthy people, the advice is to consume at least 8 glasses (8 ounces (240 mL) each) of water daily; there is little evidence to support this recommendation.(124)

Quality standards: Standards indicate that program content should address patient supports and psychosocial issues that may impede self-management. Programs should have a process for following up with patients on the status of referrals.(110)

Evidence: The Alcohol Use Disorders Identification Test (AUDIT) questionnaire's 3-item subset of questions about alcohol consumption (AUDIT-C) is a brief screening test for problem drinking that has been validated in primary care office settings.(79)

Quality standards: Standards indicate that program content should include a condition treatment plan, self-management and self-monitoring, and health behaviors (eg, substance use). Programs should have a process for following up with patients on the status of referrals.(110)

National guidelines: The US Department of Health and Human Services' Dietary Guidelines for Americans suggests that moderate alcohol consumption (2 drinks/day in men or 1 drink/day in women) may increase the overall risk of death from various causes, such as several types of cancer and some forms of cardiovascular disease.(65)

Specialty society: The American Heart Association does not support initiation of alcohol intake to improve cardiovascular health.(80)

National guidelines: The U.S. Preventive Services Task Force (USPSTF) recommends that clinicians ask all adults about tobacco use, advise them to stop using tobacco, and provide behavioral interventions and FDA-approved pharmacotherapy for cessation to nonpregnant adults who use tobacco. The USPSTF recommends that clinicians ask all pregnant persons about tobacco use, advise them to stop using tobacco, and provide behavioral interventions for cessation to those who use tobacco.(125)

National guidelines: The U.S. Preventive Services Task Force (USPSTF) concludes that there is insufficient evidence to determine the balance of benefits and harms of primary care interventions for tobacco cessation among school-age children and adolescents who already smoke because of a lack of adequately powered studies on behavioral counseling interventions and a lack of studies on medications. Clinical judgment should be used to decide how to best help youth who use tobacco.(126)

Evidence: A systematic review of 33 trials with 21,600 participants found that incentives (eg, money, vouchers) improved smoking cessation rates at long-term follow-up (at least 6 months) in mixed-population studies. Ten trials of 2571 pregnant women found that women in the incentive-rewards group were more likely to stop smoking.(127)

Evidence: A systematic review of 47 studies with more than 18,000 people showed improved smoking cessation with increased therapeutic support and nicotine replacement therapy (NRT) as compared with NRT alone.(99)

Evidence: A review of 43 studies with more than 28,000 people found with moderate certainty that when no other support is available, written self-help materials help more people to stop smoking compared with getting no help at all.(128)

Evidence: A meta-analysis of 10 randomized studies of 3655 participants using nicotine replacement therapy found that behavioral support focusing on adherence to smoking cessation medications can modestly improve adherence.(129)

Evidence: A network meta-analysis of 33 systematic reviews with 250,563 participants found that behavioral support for smoking cessation can increase quit rates at 6 months or longer, with the strongest benefit for the provision of any type of counseling and

guaranteed financial incentives.(130)

Evidence: A systematic review of 136 trials of nicotine replacement therapy (NRT) with 64,640 people found that all forms of NRT made it more likely that a person's attempt to quit smoking would succeed. Their chances of stopping smoking increased by 50% to 60% with or without additional counseling. There is no evidence that NRT increases the risk of heart attacks. In pregnant women, evidence suggests that NRT can increase the chance of quitting at the time of delivery.(131)

Evidence: A systematic review of 24 trials with 7279 participants found no evidence that adding exercise to smoking cessation support improves abstinence compared with support alone, but the evidence is insufficient to assess whether there is a modest benefit.(132)

Evidence: A systematic review of 51 studies including 22,000 people who smoked tobacco found that cessation was more likely if smokers cut down their intake when using stop-smoking medication (eg, varenicline) or fast-acting nicotine replacement therapy vs quitting all at once.(97)

Specialty Society: The American Thoracic Society recommendations for tobacco-dependent adults include using varenicline rather than a nicotine patch, using varenicline rather than bupropion, using varenicline rather than a nicotine patch in adults with a comorbid psychiatric condition, initiating varenicline in adults even if they are unready to quit, and using controller therapy for an extended treatment duration greater than 12 weeks.(102)

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Footnotes

- [A] Assess and address the emergency or urgent problems before continuing the engagement. [A in Context Link 1]
- [B] A self-care plan tells you how to take care of your condition and when and where to get help for problems. [B in Context Link 1]
- [C] A standard drink consists of 0.6 ounces (18 mL) of ethanol, which is contained in 12 ounces (360 mL) of beer (alcohol by volume (ABV) = 5%), 5 ounces (150 mL) of wine (ABV = 12%), and 1.5 ounces (45 mL) of spirits (ABV = 40%).(65) [C in Context Link 1, 2]
- [D] Electroconvulsive therapy (ECT) may be useful for severe and psychotic depression, particularly if the patient is not responsive to pharmacologic treatment.(10)(21)(133) [D in Context Link 1]
- [E] Telehealth may improve access to and coordination of mental health care. Patients with behavioral health diagnoses may use telehealth or telepsychiatry to access mental health support or care (eg, to get assistance with problem-solving techniques or ask about presenting symptoms or medication use).(134) [E in Context Link 1]
- [F] Psychosocial interventions should be individualized and involve family and other supports as necessary.(10) [F in Context Link 1]
- [G] Referral to a behavioral health specialist is based on an established treatment plan and needs at discharge from inpatient care. A psychiatrist serves as the leader of the inpatient team and can perform psychotherapy, medication prescription, and other somatic therapies. A clinical psychologist conducts individual, group, and family therapy, and administers and interprets psychological testing. A psychiatric social worker conducts individual, group, and family therapy addressing a patient's social needs to include placement, financial support, and community requirements.(29) [G in Context Link 1]
- [H] The timing of provider follow-up depends on the patient's disease severity, self-care complexity, patient/caregiver willingness and ability to provide patient care, and available social supports.(135) [H in Context Link 1]
- [I] People with chronic behavioral health conditions have higher rates of medical illness and higher mortality vs the general population. (8) [I in Context Link 1]
- [J] Medications should be regularly reviewed to eliminate medications used for acute episodes.(10) [J in Context Link 1]
- [K] Neuroleptic malignant syndrome is a rare, rapidly progressive, potentially fatal complication more common with typical antipsychotics. Onset can occur hours or years after initiation of drug therapy.(38) [K in Context Link 1]
- [L] Fall risk factors include mobility or functional impairment, poor health, weakness, incontinence, polypharmacy (4 or more medications), medication dose changes, or patient taking substances that increase risk (eg, benzodiazepines, sedative-hypnotics, antidepressants, antipsychotics, alcohol, opiates).(61)(136) [L in Context Link 1]
- [M] A pharmacist can support medication reconciliation for redundancy or duplication and identify drug interactions (eg, with other drugs, food).(58) [M in Context Link 1]
- [N] Services to consider for community support include stairway modifications, widening of interior doors for ambulation equipment, and installation of handrails.(50) [N in Context Link 1]
- [O] Adequate intake is used when the recommended daily allowance cannot be determined because of insufficient data on requirements.(66) [O in Context Link 1]
- [P] Primary pharmacotherapy for tobacco dependence includes varenicline, bupropion, nicotine gum, nicotine inhaler, nicotine nasal spray, and nicotine patch. Nicotine replacement therapies (NRTs) should be used with caution for patients in the 2 weeks following myocardial infarction and for those with life-threatening arrhythmias or severe angina.(94)(125) [P in Context Link 1]

Codes

ICD-10 Diagnosis: F06.33, F30.10, F30.11, F30.12, F30.13, F30.2, F30.3, F30.8, F30.9, F31.0, F31.10, F31.11, F31.12, F31.13, F31.2, F31.30, F31.31, F31.32, F31.4, F31.5, F31.60, F31.61, F31.62, F31.63, F31.64, F31.70, F31.71, F31.73, F31.75, F31.77, F31.81, F31.89, F31.9 [Hide]

DSM-5: F06.33, F31.0, F31.11, F31.12, F31.13, F31.2, F31.31, F31.32, F31.4, F31.5, F31.71, F31.73, F31.75, F31.81, F31.89, F31.9 [Hide]

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